INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Everolimus · neuroendocrine tumours

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00320
Version 6

Daily treatment, with four-weekly safety blood tests

Treatment stages

01 **Daily everolimus**
Continuous

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for new or worsening cough, breathlessness or fever, mouth ulcers limiting eating or drinking, marked thirst, much less urine, severe diarrhoea or signs of infection. Severe breathing difficulty or collapse needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

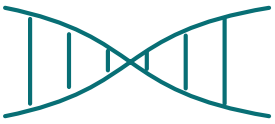
How your medicines work

Everolimus is a daily targeted tablet used to help control particular advanced neuroendocrine tumours (NETs). It aims to slow growth; the team balances disease control, side effects and your priorities. This neuroendocrine tumour (NET) guide does not cover the separate kidney-cancer indication.

TARGETED THERAPY TABLET

Everolimus

Afinitor



Reduces mTOR growth signalling

Everolimus blocks a growth-control pathway called mTOR inside cells. This can slow NET growth. Healthy tissues and immune responses also depend on these pathways, helping explain mouth ulcers, infections, lung inflammation and changes in blood sugar or fats.

mTOR signal → Growth pathway blocked → Tumour growth may slow

What to watch for

Report mouth ulcers and new breathing symptoms early; keep blood and metabolic checks even if you feel well.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your everolimus treatment

Daily treatment with regular safety reviews.

The exact Irish NET indication

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c. The gastrointestinal (GI) indication is nonfunctional; do not transfer it to lung NET or renal cell carcinoma.

Trial evidence and limits

Everolimus delayed progression compared with placebo in these trial populations. That does not mean cure or predict an individual response. The studies do not provide one personal survival estimate. RADIANT-3 final overall survival did not show a statistically significant difference, with crossover complicating interpretation. RADIANT-4 combined GI and lung populations; its inclusion of lung NET does not broaden NCCP 00320c.

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour begins in cells with hormone-making features. Its starting organ and the places it has spread are separate parts of the diagnosis. A pancreatic neuroendocrine tumour differs from pancreatic adenocarcinoma; a deposit in the liver may have come from another organ.

Differentiation, grade and Ki-67

The pathology report describes how organised the cells look (differentiation) and their growth activity (grade). Ki-67 measures cells showing a growth-related protein in the sample. Its percentage describes that sample, not the amount of your body affected or your chance of survival.

Grade is different from stage

Stage describes extent; grade describes tumour biology. Either can vary independently. A well-differentiated grade 3 neuroendocrine tumour and a poorly differentiated neuroendocrine carcinoma are distinct diagnoses and may need different treatments.

Functioning and nonfunctioning

A functioning tumour produces a hormone syndrome. A nonfunctioning tumour can still cause problems through growth or pressure. Somatostatin receptor uptake on a scan shows a treatment target; it does not, on its own, identify a hormone syndrome.

Your diagnosis record

Ask for one diagnosis summary: starting organ, grade, differentiation, Ki-67 result if reported, sites involved, hormone syndrome and receptor-imaging result. Keep pending results clearly marked.

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Agree your treatment aim: slowing growth, reducing tumour size, controlling hormone symptoms or preserving daily function. A scan showing no growth may be a useful result even when the tumour has not shrunk.

How treatments fit together

Your plan may use different treatments at different times, including surgery, medicines, receptor-targeted radiation or treatment directed at the liver. Ask the team to mark your own sequence and who coordinates it.

Choosing between options

Ask why the proposed option fits the tumour site, grade, receptor findings, symptoms and your health. Discuss reasonable alternatives, observation when appropriate and suitable trials. Your team explains which options are available to you.

Evidence is about groups

Studies measure different outcomes, such as shrinkage, time before worsening and survival. Ask which outcome matters for your plan and how closely the study group matches your diagnosis. No study average can predict one person’s course.



Review points and shared decisions

Agree when to review the plan and what could lead to continuing, pausing or changing treatment. Include the effects on work, caring duties, travel and the activities that matter to you.

Evidence depends on the NET site and setting

The principal trials assessed time before disease worsened.

CHAPTER 02

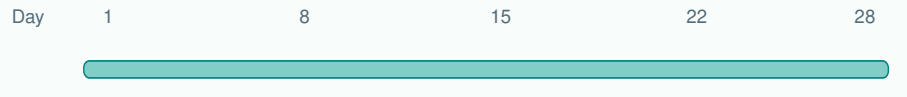
Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

How your treatment fits together

Everolimus · neuroendocrine tumours 28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CONTINUOUS

Daily everolimus

Daily | 4-week checks

Continue daily only while the team confirms benefit and tolerability. Blood reviews do not create a planned tablet break.

The written prescription and team review determine treatment; this timeline is a reference.

BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, NET response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00320 v6 · current regimen · EMA product information and patient leaflet

Tests in your plan

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory's food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Everolimus · neuroendocrine tumours: treatment and checks

Take the prescribed everolimus dose once daily every day, at the same time and consistently with or without food. There is no planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00320	full blood count (FBC), renal/liver tests, blood urea nitrogen (BUN) and urine protein	FBC, kidneys and liver every 4 weeks	Earlier assessment if unwell, dehydrated or less urine	Review abnormalities before continuing	Follow unresolved effects
Glucose / blood fats NCCP 00320	Fasting glucose, cholesterol and triglycerides	Lipids regularly; fasting glucose as clinically indicated	Report thirst, frequent urine or marked fatigue	Review treatment for diabetes or lipid changes	Individual follow-up
Infection / mouth / lungs NCCP 00320	Review active infection; hepatitis B surface antigen and core antibody	Ask about ulcers, cough, breathlessness, infection and rash	New breathing or infection symptoms need prompt contact	Assess lung inflammation versus infection	Follow symptoms after a hold or stopping
NET response / hormone plan NCCP 00320	Document tumour site, grade, symptoms and baseline assessment	Scans / markers only as individually scheduled	Track symptoms and separate hormone medicines	Review benefit, burden and personal goals	Agree next treatment and follow-up

What each check is for

1 **Daily everolimus**

Continuous

Everolimus is a daily targeted tablet used to help control particular advanced NETs. It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

Ask / act: Confirm the current prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Taking tablets

The usual starting dose is 10 mg once daily, but follow your own prescription. Swallow whole with a glass of water; do not chew or crush. Take consistently with or without food. If a dose is missed, do not take an extra dose: take the next prescribed dose at the usual time.



STEP 2

Medicines, vaccines and food

Avoid grapefruit and St John’s wort. Ask the pharmacist to check all prescribed, over-the-counter and herbal products. Live vaccines should be avoided during treatment; agree vaccination with your team.



STEP 3

Procedures and radiotherapy

Discuss planned surgery, wounds and current or recent radiotherapy. Everolimus may affect healing and can worsen or reactivate radiation-related inflammation. The teams agree any interruption.



STEP 4

Safe storage and supply

Keep tablets in their original labelled packaging, out of children’s reach, and follow the pack storage instructions. Return unused tablets to the pharmacy as advised. Plan repeat prescriptions before your supply runs out.

STEP 5

Pregnancy, feeding and fertility

Tell the team before starting if pregnant, planning pregnancy or breastfeeding. Ask for the medicine-specific contraception duration and fertility advice, and record it in your plan.

STEP 6

Review visits

Baseline blood count, kidneys/liver, BUN, urine protein, lipids, fasting glucose and hepatitis B screening. Blood count, kidney and liver tests every four weeks. Lipids regularly and glucose as clinically indicated; scans and NET markers follow the consultant plan.

Daily tablets, checks and mouth care

Keep the current prescription with your treatment record.

Taking tablets

Medicines, vaccines and food

Tests and review

Mouth care

Use gentle mouth care and any prescribed alcohol-free mouthwash. Ask before buying a rinse; avoid products containing alcohol, peroxide, iodine or thyme. Report ulcers early.

Sickness and bowel support

Everolimus has minimal-to-low emetogenic potential. Routine premedication is not required by this regimen; agree any rescue anti-sickness or diarrhoea medicine with the team.

Infection prevention

Existing infection must be assessed and resolved before starting. Hepatitis B testing informs any specialist prevention plan. Vaccination and any antimicrobial prevention, including Pneumocystis jirovecii pneumonia (PJP) prevention with certain steroid plans, are individual decisions.

Eating and energy

Report weight loss, poor intake or persistent fatigue. Ask for nutritional and symptom support; do not continue through severe symptoms simply to finish a pack.

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts assess oxygen-carrying cells, infection-fighting cells and platelets. Kidney and liver results help assess treatment safety. These checks are needed even when you feel well; a reassuring scan answers a different question.



computed tomography (CT), magnetic resonance imaging (MRI) and receptor imaging

CT and MRI show anatomy and tumour size. Somatostatin receptor imaging tests whether a particular receptor target is present. An fluorodeoxyglucose positron emission tomography (FDG-PET) scan measures uptake of a glucose-like tracer and is used selectively. Your team chooses the scans that can guide your care.

Hormones and tumour markers are individual

Selected markers may help answer a hormone or tumour question. Examples include 5-hydroxyindoleacetic acid (5-HIAA) for serotonin-related symptoms, chromogranin A, or a specific pancreatic hormone. They are not a routine panel for every neuroendocrine tumour. Interpret changes with symptoms and imaging.

Preparing for a test

Use the laboratory instructions for fasting, food restrictions and any timed urine collection. Some medicines affect marker results, including acid-suppressing medicines and chromogranin A. Ask the team what to do; do not stop a prescribed medicine yourself.

Heart and inherited-risk checks

Some hormone syndromes can affect heart valves, so an ultrasound of the heart may be requested. Inherited-risk assessment depends on the tumour and family history; multiple endocrine neoplasia type 1 is one example. An inherited-risk test and a test of tumour tissue answer different questions.

Who does what, and when?

For each test, write its name, purpose, booking date and result-review contact. Ask the team to set your next scan or marker date. A schedule for someone after surgery may not apply during your treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Everolimus every day	 **Days 1–28** Continue into the next review window unless your team tells you to hold.
Mouth, breathing and infection watch	 **Days 1–28** Report changes when they happen.
Four-week safety review	 **Day 28** Blood count, kidneys and liver; other checks on your individual plan.

There is no planned rest week: The 28-day visual is a review window. Do not take a routine week off or continue through a team-directed hold.

Monitoring continues after changes: Ask who follows unresolved symptoms or abnormal tests after a delay, dose change or stopping.

Lung effects and infection can look similar: A new cough or breathlessness needs assessment; do not assume it is a harmless tablet effect or start leftover antibiotics or steroids yourself.

> **Why these bands stay broad:** Treatment timing is shown for orientation. Symptoms can develop on any day; no symptom band predicts an individual toxicity peak. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00320 v6 · current regimen ↗

> **There are no guaranteed "safe days".** Follow your oncology unit's emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Between-treatment notes

Day 1–7: Starting safely

Take the prescribed daily dose consistently with or without food. Start the mouth-care plan and check other medicines. Confirm the first blood-test date and who to call.

Day 8–28: Continue daily; report symptoms early

Mouth, breathing, infection, bowel or skin symptoms can occur at any time. Do not wait for the four-week review. Attend blood checks and follow any written hold or dose-change instructions.

When to contact your team

Do not wait for the next scheduled blood test.

Dose holds and restarting

The team may hold or reduce treatment for side effects or abnormal tests. Record the reason and the new plan. Do not restart a held dose or increase a reduced dose yourself.

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Keep a brief record of your usual bowel pattern, flushing, appetite, weight, pain and energy. Note new changes and when they started. This helps the team assess whether treatment, hormones or another problem needs attention.

Hormone-related symptoms

Hormone symptoms depend on the tumour: examples include flushing and watery diarrhoea, acid-related symptoms, or changes in blood sugar. An insulin-producing tumour can lower blood sugar. Ask which symptoms apply to you and whether you need a written rescue plan.

Hormone treatment is a separate part of the plan

Hormone-control medicines have their own schedule. A pause in anticancer tablets does not automatically mean pausing those medicines. Before peptide receptor radionuclide therapy, the specialist team coordinates any interruption and restart of somatostatin analogue injections.

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Procedures and hormone syndromes

Tell teams arranging an operation, anaesthetic or other procedure about a hormone-producing tumour or previous hormone crisis. Your specialist decides whether extra preparation is needed. Carry the agreed emergency instructions and current medicine list.

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.

Food, weight and digestion

Report weight loss, persistent diarrhoea, greasy stools or difficulty eating. Different causes need different treatment. Ask for a dietitian and whether reduced digestive-enzyme function needs assessment.

Fluids and safe activity

Use your personal fluid advice, especially with diarrhoea or heart or kidney disease. Adjust activity and rest to your energy. Report dizziness, breathlessness or fatigue that limits daily tasks so treatable causes can be checked.

Emotions, relationships and quality of life

Ask for help with mood, sleep, relationships, intimacy or caring responsibilities when you need it. Symptom and palliative-care teams can support comfort and quality of life while anticancer treatment continues.

Practical help

Ask your specialist nurse or social worker about transport, employment, finances and help at home. Tell them about caring responsibilities and ask which support organisations fit your needs.

A record that works without a phone

Keep a paper schedule, current medicine list and real contact numbers together. The handbook can be read without internet access. Use the written emergency plan whenever you need urgent help.

Questions to take to the next visit

For the next visit, choose the questions that matter most: my exact diagnosis; the aim of treatment; which tests I need and why; who explains results; when to call; and what help is available.

Questions worth asking

Which NET indication and treatment aim apply to me?

Ask your team to name the exact pancreatic or nonfunctional GI NET indication, the treatment aim and how it fits with your previous care. This guide does not cover the separate kidney-cancer indication.

When are my four-week blood tests and glucose/lipid checks?

NCCP specifies blood count, kidney and liver tests every four weeks. Lipids are checked regularly and fasting glucose as clinically indicated. Write your actual test dates and result-review contact in the passport.

What mouthwash or rescue medicines have you prescribed?

Ask the team to write the mouth-care and any rescue medicine prescription. Alcohol-free mouthwash may be required; there is no automatic routine anti-sickness premedication in this regimen.

What should I do about a new cough, fever or breathlessness?

Contact oncology promptly. Infection and non-infectious lung inflammation can look similar and need assessment. Severe breathing difficulty or collapse needs emergency help.

How does this fit with my hormone-control treatment?

Ask the NET team to list any separate hormone-control medicine and its schedule. Do not stop injections or other prescribed treatment simply because everolimus is held.

When will we review scans, symptoms and my priorities?

The consultant sets the imaging and marker plan and reviews benefit, side effects and your goals. Ask for the next decision date and who will explain the results.

> **Try explaining the plan back:** Can you explain the daily tablet plan, the four-week blood checks, mouth care and which breathing or infection symptoms need prompt contact?

Your neuroendocrine tumour treatment Progressive unresectable or metastatic pancreatic neuroendocrine tumour (NET), or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous daily everolimus · NET

Active medicines: Everolimus

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Use gentle mouth care and any prescribed alcohol-free mouthwash. Ask before buying a rinse; avoid products containing alcohol, peroxide, iodine or thyme. Report ulcers early.
- Everolimus has minimal-to-low emetogenic potential. Routine premedication is not required by this regimen; agree any rescue anti-sickness or diarrhoea medicine with the team.
- Existing infection must be assessed and resolved before starting. Hepatitis B testing informs any specialist prevention plan. Vaccination and any antimicrobial prevention, including Pneumocystis jirovecii pneumonia (PJP) prevention with certain steroid plans, are individual decisions.
- Report weight loss, poor intake or persistent fatigue. Ask for nutritional and symptom support; do not continue through severe symptoms simply to finish a pack.
- No routine premedication is required by NCCP. No universal steroid, antimicrobial or growth-factor prescription is implied. Mouthwash and rescue treatment follow the individual prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / eating

Everolimus

Mouth ulcers may appear early. Use prescribed mouth care; report soreness, pain or difficulty eating and drinking.

3 Blood / infection

Everolimus

Blood counts can fall. Fever, shivering, unusual bruising or sudden illness needs urgent advice even without a scheduled visit.

5 Kidneys / fluid

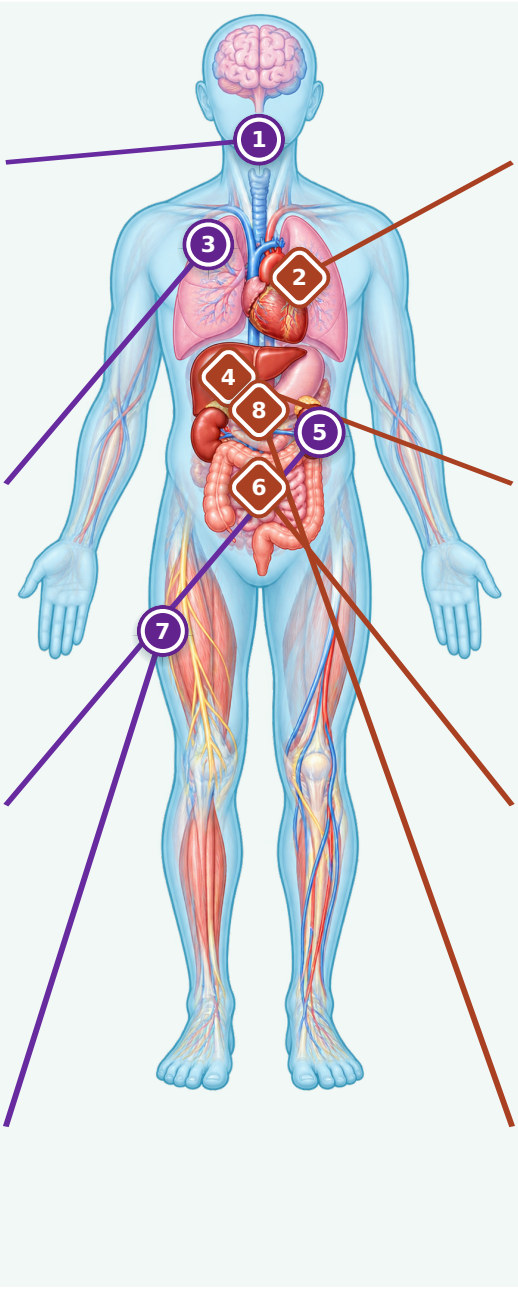
Everolimus

Kidney problems or protein in urine may need extra tests. Report swelling, dehydration or much less urine.

7 Skin / wounds

Everolimus

Rash and slow wound healing can occur. Report severe rash or unhealed wounds and discuss surgery before it is arranged.



2 Lungs / breathing

Everolimus

New cough, fever or breathlessness may be infection or lung inflammation. Contact oncology promptly; severe breathlessness is an emergency.

4 Liver

Everolimus

Tests can change before symptoms occur. Attend blood checks and report yellow skin or eyes, dark urine or persistent abdominal pain.

6 Bowel / appetite

Everolimus

Diarrhoea, nausea or poor appetite can affect fluids and weight. Report persistent symptoms or inability to drink.

8 Blood sugar / energy

Everolimus

Blood sugar and fats may rise without symptoms. Report marked thirst, frequent urine or unusual fatigue; keep the agreed tests.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology promptly for new or worsening cough, breathlessness or fever, mouth ulcers limiting eating or drinking, marked thirst, much less urine, severe diarrhoea or signs of infection. Severe breathing difficulty or collapse needs emergency help.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Use the prescribed local supportive-care plan; independent clinical/pharmacy review pending**
   https://healthservice.hse.ie/documents/6718/320_Everolimus.pdf
2. **EMA product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/afinitor-epar-product-information_en.pdf
3. **NCI: gastrointestinal neuroendocrine tumours — clinical evidence**
   https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq
4. **NCI PDQ: pancreatic neuroendocrine tumours (NETs) · hormone and digestive background**
   https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq
5. **RADIANT-3 primary pancreatic neuroendocrine tumour (NET) report**
   https://pubmed.ncbi.nlm.nih.gov/21306238/
6. **RADIANT-3 final overall-survival analysis**
   https://pubmed.ncbi.nlm.nih.gov/27621394/
7. **RADIANT-4 nonfunctional gastrointestinal (GI)/lung NET trial**
   https://pubmed.ncbi.nlm.nih.gov/26703889/
8. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
9. **NANETS expert consensus: high-grade neuroendocrine neoplasms, 2023**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10388681/
10. **NCI: gastrointestinal neuroendocrine tumours — patient information**
    https://www.cancer.gov/types/gi-neuroendocrine-tumors/patient/gi-neuroendocrine-treatment-pdq
11. **NCI: eating and nutrition during cancer treatment**
    https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
12. **NCI: palliative care during cancer treatment**
    https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
13. **NCI: pancreatic neuroendocrine tumour diagnosis and tests**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors/diagnosis
14. **NCI: pancreatic neuroendocrine tumour symptoms**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors/signs-symptoms
15. **NCI: pancreatic neuroendocrine tumour treatment**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors/treatment
16. **NCI: pancreatic neuroendocrine tumour types**
    https://www.cancer.gov/types/pancreatic/neuroendocrine-tumors